

HOW TO OPEN AN IN-HOME CARE AGENCY in Oregon

An OHA-licensed In-Home Care Agency (Limited/non-medical) — mostly private pay.

A complete, plain-language, step-by-step startup guide — written so you can actually DO each step.

Figures current for 2026 · Every contact, rate & fee should be re-verified before you rely on it.

The Oregon model — read this first

In Oregon, any agency that sends caregivers into people's homes to help with daily living must be licensed as an In-Home Care Agency by the Oregon Health Authority (OHA), under ORS 443.305–443.350 and OAR chapter 333, Division 536. Oregon uses one license with four classifications that scale from non-medical to nursing — a non-medical personal-care agency applies for the “Limited” (or “Basic”) classification and does NOT need a nurse on staff. One important business reality: Oregon's Medicaid personal care flows mostly to individual “homecare workers” the client hires (through the Home Care Commission), not to agencies — so an Oregon in-home care agency is largely a PRIVATE-PAY business. This guide walks the whole path, broken down so a first-timer can follow it.

Oregon's four classifications — pick “Limited” for non-medical. One In-Home Care Agency license, four tiers (OAR 333-536-0007): *Limited* = personal care only (may include medication reminding) — the non-medical tier; *Basic* = adds medication assistance; *Intermediate* = adds medication administration; *Comprehensive* = adds nursing (requires an RN on staff). A non-medical agency applies for Limited or Basic and needs no nurse — simpler and cheaper. You can move up a tier later.

Know where the Medicaid money goes. Oregon funnels most Medicaid personal care to individual homecare workers (paid through the Oregon Home Care Commission) and consumer-directed models — NOT to agencies. Agencies CAN contract with the state (APD) to provide in-home services, but it's the minority path and low-margin. So build your business on private pay, long-term-care insurance, and the VA first (Part 8).

How to use this guide

Read it once start to finish, then use it as a checklist and a phone list. It's written for someone who has never opened an agency — where we name a thing, we break it down and tell you exactly where to go and what to ask. It is general information, not legal, tax, or clinical advice; verify each step with OHA and qualified counsel (see Part 11) before you rely on it.

The journey at a glance

1	Decide this is your business	A licensed in-home care agency — recurring, hourly revenue (Part 1)
2	Form the business & insure	Entity, EIN, NPI, insurance & bonding (Part 3)
3	Get the OHA license (Limited)	Application, administrator, the fee, the survey (Part 4)
4	Set up your administrator	2 years health-related experience (Part 4)
5	Recruit & screen caregivers	Orientation, competency, background check (Part 5)
6	Set up operations	Policies, scheduling, systems (Part 6)
7	Serve private-pay clients	Your core market — start on your license (Part 8)

8	Add VA & LTC insurance	VA Community Care + LTC-insurance clients (Part 8)
9	Get your first clients	Discharge planners, the ADRC/AAAs, care managers (Part 10)
10	Who to call & what to ask	Every contact + the exact questions, in one place (Part 11)

Rule of thumb. Apply for the Limited classification, build your private-pay book, and add the VA and LTC insurance. Oregon requires a pre-licensure survey, so build your policies and files to standard before you apply, and expect a multi-month process.

PART 1

What This Business Is

In short: An OHA-licensed In-Home Care Agency (Limited/non-medical) — caregivers who help clients with daily living in their own homes. Personal care, homemaker, companionship; mostly private pay.

Oregon licenses In-Home Care Agencies to provide services in a client's home. The Limited classification covers personal care — help with the activities of daily living (bathing, dressing, grooming, toileting, transferring, eating, mobility) — and may include medication reminding, plus homemaker and companion help. It does not include medication administration or nursing (those are higher classifications).

The four classifications

Classification	What it adds	Nurse needed?
Limited	Personal care only (may include med reminding)	No
Basic	+ medication assistance	No
Intermediate	+ medication administration	No (but extra policies)
Comprehensive	+ nursing services	Yes — RN on staff

Key terms. OHA = Oregon Health Authority (your licenser). HCRQI = Health Care Regulation & Quality Improvement (the OHA program). ORS 443 / OAR 333-536 = the law and rules. APD = Aging & People with Disabilities (Medicaid). OHCC = Oregon Home Care Commission (runs the individual-homecare-worker system). ICP = Independent Choices Program (consumer-directed). CCO = Coordinated Care Organization (Oregon Health Plan managed care). eXPRS = Oregon's Medicaid EVV/payment system. BCU = the Background Check Unit. NPI = National Provider Identifier.

Why this business — and why now

Demand for in-home care is rising fast, and for reasons that aren't going to reverse: people overwhelmingly want to age in their own homes rather than move to a facility; the 65-and-over population grows every year; and hospitals now discharge patients “quicker and sicker,” which sends more families looking for help at home right after a hospital stay. Oregon's 65-and-over population is among the fastest-growing in the country and the state's strong “age in place” culture keeps demand for in-home help climbing faster than facilities can absorb it — a private-pay tailwind for a well-run agency. And it's a business you can start lean — there's no building to buy or fill, the revenue is recurring and hourly, and a good reputation compounds through referrals.

The three mistakes that sink new agencies — avoid these. New home-care owners rarely fail because of the care itself. They fail for three business reasons: (1) undercapitalization — they run out of cash covering weekly payroll before Medicaid or insurers reimburse (see the cash-flow note in Part 9); (2) weak caregiver recruiting and retention — they can't reliably staff the shifts they're offered, so referral sources quietly stop calling; and (3) sloppy compliance — a missing background check, training record, or (for Medicaid) EVV visit that triggers a payback or a survey finding. Get your cash runway, your staffing pipeline, and your paperwork right, and the rest follows.

PART 2

Who Regulates You (and What Each One Does)

In short: OHA licenses and surveys you, ODHS/APD and the Home Care Commission run the (mostly individual-provider) Medicaid side, and the Background Check Unit screens your people.

OHA — Health Care Regulation & Quality Improvement (your licensor)

OHA's HCRQI program issues your In-Home Care Agency license, writes the OAR 333-536 rules, reviews your application and policies, and conducts the pre-licensure survey and ongoing oversight. Phone 971-673-0540; mailbox.inhomecare@odhaoha.oregon.gov; healthoregon.org/hflc.

ODHS/APD & the Home Care Commission (Medicaid)

Oregon's Medicaid long-term in-home care runs mostly through Aging & People with Disabilities and the Oregon Home Care Commission — which supports the individual homecare workers that clients hire and direct. Agencies can contract with APD to provide in-home services (OAR 411-033), but most hours flow to homecare workers, not agencies.

The Background Check Unit (BCU)

Owners/administrators with direct client contact and your caregivers go through the ODHS/OHA Background Check Unit / Clearinghouse (OAR 407-007) for a fitness determination before serving clients.

PART 3

Form Your Business & Insure — Step by Step

In short: *This is where most first-timers get stuck. We break each piece all the way down — what it is, where to go, exactly what to do, and what to have in hand.*

3.1 Form your legal entity (an LLC)

An “entity” is the legal business — almost always a Limited Liability Company (LLC) — that holds your license or registration, your contracts, and your bank account, and that shields your personal assets if the business is ever sued. In Oregon you form the LLC through the Secretary of State's Corporation Division, then register for payroll taxes with the Department of Revenue (an Oregon BIN) so you can hire employees. You'll name a “registered agent” (a person or service with a physical in-state address who receives legal mail — this can be you) and adopt an Operating Agreement (the internal document that sets out who owns what and who can make decisions).

- 1. File your Articles of Organization.** Through the Secretary of State's Corporation Division at sos.oregon.gov — the filing fee is \$100.
- 2. Name a registered agent and adopt an Operating Agreement.** The agent (which can be you) receives legal mail at an Oregon street address; the Operating Agreement sets ownership and management.
- 3. Get your Oregon BIN.** Register for payroll withholding (a Business Identification Number) with the Oregon Department of Revenue before you run payroll.
- 4. Calendar your annual report.** Oregon LLCs file an annual report — \$100 — by the anniversary of formation to stay in good standing.

No newspaper publication in Oregon. *Unlike some states, Oregon has no LLC publication requirement — forming the entity is a straightforward filing plus your payroll registration.*

3.2 Get your EIN — the free federal tax ID

An EIN (Employer Identification Number) is your business's federal tax number — think of it as a Social Security number for the company. It's free, takes about ten minutes, and you need it before you can open a bank account, run payroll, or enroll with any payer. Here's exactly how to get it:

- 1. Go to IRS.gov and open the EIN Assistant.** Search “Apply for an EIN online.” Apply directly on the IRS website — never pay a third-party site that charges for this free service.
- 2. Choose your entity and enter the responsible party.** Select “Limited Liability Company,” then give the responsible party's legal name and SSN or ITIN (usually you, the owner), plus your business name and address.
- 3. Finish in one sitting.** The online session times out after about 15 minutes of inactivity and can't be saved partway — have your details ready before you start.
- 4. Save your EIN and the CP-575 letter.** You get the EIN on the spot and can download the official CP-575 confirmation letter — save both; your bank and every payer will ask for them.

3.3 Get your NPI — the national provider ID

An NPI (National Provider Identifier) is a free, ten-digit ID number that health-care payers use to identify your organization. You'll use it if you contract with the VA or with Oregon Medicaid (APD) to bill for in-home services. Your AGENCY needs a Type 2 (Organizational) NPI — which is different from the Type 1 (individual) NPI a single person would get.

- 1. Create an I&A account.** At nppes.cms.hhs.gov, set up an Identity & Access (I&A) account for whoever will manage the number — usually an owner.
- 2. Start a Type 2 (Organizational) application.** Log into NPPES and choose Type 2, not Type 1. Type 1 is for an individual person; your business is a Type 2 organization.

3. Enter your legal name, EIN, and address, and pick your taxonomy. Choose the taxonomy that fits a home-care / personal-care agency. The “taxonomy” is the code that says what kind of provider you are — confirm the exact one your Medicaid program wants before you submit.

4. Submit — it's free and quick. It usually processes in about 10 business days. Save the NPI; you'll use it on every enrollment and claim.

3.4 Open a business bank account

Keep the business's money completely separate from your personal money — it protects your liability shield and makes payroll, taxes, and payer deposits clean. Open the account once your entity and EIN are set up.

What to bring to the bank:

- Your EIN confirmation (CP-575) from the IRS
- Your filed Articles of Organization / Certificate of Formation and the entity ID number
- Your signed Operating Agreement
- A Beneficial Ownership form (the bank collects details on anyone owning 25%+ plus a control person)
- A government photo ID, SSN, date of birth, and home address for each owner/signer
- An opening deposit (often around \$100)

Where to go: a bank with lots of Oregon branches — Umpqua/Columbia Bank, U.S. Bank, KeyBank, Wells Fargo, or a local credit union like OnPoint or Oregon Community Credit Union. Home care is payroll-heavy — you pay caregivers weekly, but Medicaid and insurers pay you weeks later — so ask specifically about a business line of credit or invoice financing to bridge payroll before you scale.

3.5 Get your insurance — what you need, who sells it, and what to ask

A home-care agency sends employees into clients' homes, so you need coverage built for that risk. Buy it before you serve your first client:

Coverage	What it protects against	Must-have?
General & Professional Liability	A slip-and-fall or property damage in a client's home, plus claims about the CARE (a caregiver's error or neglect)	Yes — foundational
Abuse & Molestation	Allegations of abuse by a caregiver — general liability EXCLUDES this	Yes — high exposure
Employee Dishonesty / Bond	Theft from a client's home — families ask if caregivers are “bonded”	Yes — clients & payers expect it
Non-Owned & Hired Auto	Your caregivers driving their own cars to clients or with clients	Yes — critical for home care
Workers' Compensation	Caregiver injuries (lifting/transferring)	Required in Oregon for employees

Real places to get it

Buy through an independent agent or broker who writes Oregon home-care programs. Carriers and programs active in this niche include Philadelphia Insurance (PHLY), CNA, The Hartford, Markel, and home-care specialty programs from Caitlin Morgan and NIP Group / HomeCareUSA. To find a local Oregon agent, use [trustedchoice.com](https://www.trustedchoice.com) (filter for home care). Confirm the carrier writes non-medical home care in Oregon before you rely on a quote.

What to ask the insurance agent

“Do you write Oregon In-Home Care Agencies — which carriers, and what's the minimum premium?”

“Is abuse & molestation included or a sub-limit, and do you include a dishonesty bond?”

“Do you include non-owned & hired auto (my caregivers drive their own cars)?”

“What coverage limits do the VA or long-term-care insurers require?”

“What will Oregon workers' compensation cost given my caregiver payroll?”

PART 4

Get Your OHA License — Step by Step

In short: Assemble your application, administrator, and policies; pay the fee; and pass the pre-licensure survey. Apply for the Limited classification for non-medical care.

4.1 Before you apply

- ☐ Entity formed; EIN and NPI obtained; insurance & bond bound
- ☐ An administrator with at least 2 years of professional/management experience in a health-related field (plus HS diploma)
- ☐ Your three required policy sets: organization; disclosure/screening; service plans
- ☐ Background-check request forms for owners/administrators with direct client contact

4.2 The application & survey

- 1. Submit the In-Home Care application.** Choose the Limited (or Basic) classification; include your administrator application and résumé and the non-refundable fee. From OHA HCRQI (healthoregon.org/hflic).
- 2. Pay the fee.** On the statutory schedule, Limited is about \$2,000 initial / \$1,000 renewal (Basic \$2,250 / \$1,000). Confirm the current amount on OHA's live application.
- 3. Pass the pre-licensure survey.** After your initial steps are complete, OHA schedules an on-site survey before the license issues.
- 4. Receive your license.** Then renew annually. (Note: a referral/matchmaker service that schedules caregivers, sets pay, or places workers is pulled into licensure too — you can't dodge the license with a registry model.)

4.3 Timeline

OHA doesn't publish a fixed timeframe and notes application volume is high — plan for a multi-month process. Build your policies and files to standard so your survey goes smoothly, and line up private-pay referrals to start the day you're licensed.

PART 5

Your Administrator & Caregivers — by Position

In short: A Limited agency needs no nurse. Your administrator and your caregiver competency program are the core requirements. Here they are, one role at a time.

5.1 Your administrator

The administrator must have at least two years of professional or management experience in a health-related field, plus a high-school diploma or equivalent. This can be you. A Limited/non-medical agency does NOT need an RN (an RN is required only for the Comprehensive/nursing classification).

5.2 Your caregivers — orientation & competency (OAR 333-536-0070)

- **Age & communication:** at least 18, with sufficient communication/language skills.
- **Orientation before working independently:** duties, clients' rights, ethics, infection control, emergency response, and (if applicable) medication reminding.
- **Competency-based training:** before independent service — recognizing and responding to medical emergencies, adverse behaviors, and nutrition/hydration — evaluated and documented by the administrator or a designee.
- **Annual education:** at least 6 hours related to caregiver duties each year (plus 1 more hour if doing medication administration).

5.3 Background checks & health

1. **Run the background check.** Through the ODHS/OHA Background Check Unit / Clearinghouse (OAR 407-007) — a fitness determination before serving clients, for caregivers and for owners/administrators with direct client contact.
2. **Confirm the TB expectation.** OAR 333-536's caregiver rule doesn't state a specific TB test — confirm the current personnel/infection-control expectation with OHA and follow public-health guidance.
3. **Document everything.** Keep the background determination and competency records in each caregiver file.

5.4 Recruiting and keeping good caregivers — the real make-or-break

Every experienced agency owner will tell you the same thing: your business rises or falls on caregivers, not clients. Referral sources will send you all the clients you can handle — but only if you can reliably staff them. So treat recruiting and retention as your core operation, not an afterthought.

- **Recruit constantly:** post continuously (Indeed, local Facebook groups, word of mouth, and a caregiver-referral bonus), and make applying fast — the best caregivers take the first agency that calls them back and schedules an interview.
- **Hire for reliability and warmth:** specific skills can be trained; showing up on time and being kind cannot. Check references for dependability, not just experience.
- **Onboard properly:** a real orientation, a skills check-off before a caregiver works alone, and a written scope of what they may and may not do — this protects clients and builds the caregiver's confidence.
- **Retain deliberately:** consistent hours, quick and correct pay, being reachable when a caregiver has a problem, genuine recognition, and a small raise ladder. Turnover is the biggest hidden cost in home care — every caregiver who quits costs you recruiting, training, and often the client they served.

PART 6

Policies, Operations & Systems

In short: Your OHA policies are your operating backbone. Add scheduling and billing systems. Because you're mostly private pay, you generally won't need Medicaid EVV unless you contract with APD.

6.1 Policies

Build your policy manual to OAR 333-536: organization; disclosure and screening; service plans; client rights; caregiver competency; complaint handling; incident and abuse reporting; emergency preparedness; and infection control. This is what your surveyor reviews.

6.2 EVV (only if you bill Medicaid via APD)

Oregon's Medicaid EVV runs on the state-built eXPRS system. If you contract with APD to bill Medicaid, you'll use eXPRS; for a private-pay agency, EVV generally doesn't apply. Pick software that can support it if you plan to add Medicaid.

6.3 Systems

Choose a home-care platform for scheduling, caregiver/client records, visit tracking, invoicing, and payroll.

6.4 Documentation & staying inspection-ready

Whatever your state requires, the agencies that pass surveys and payer audits cleanly all do the same thing: they keep complete, current files from day one instead of scrambling the week before an inspection. Build these habits now, because reconstructing records after the fact is nearly impossible:

- **A file for every caregiver:** the application, background checks, training and competency records, health/TB screening, and a signed acknowledgment of your policies — all dated and kept current.
- **A file for every client:** the assessment, the service/care plan, the signed service agreement, consents, ongoing visit notes, and any payer authorizations.
- **A visit record for every shift:** who worked, when they arrived and left, and what care was provided — and, for Medicaid, the matching EVV record.
- **An incident and complaint log:** every incident, injury, and complaint and exactly how you resolved it — inspectors look for a real, used quality-improvement process, not a binder that's never been opened.

PART 7

The Client's Journey — Intake to Discharge

In short: A client is referred (usually privately), you assess and set up the service plan, a caregiver delivers the hours, you supervise, and you review and adjust.

Stage 1 — Referral & inquiry

A referral comes from a family, a hospital discharge planner, a home-health/hospice partner, or an ADRC/case manager (Part 10). Take the basics — who, what tasks, how many hours, where, when, and who pays (private, VA, LTC insurance, or APD).

What you confirm on the first call:

- Who needs care and what tasks; how many hours and when to start; the address and service area; who pays; and that the need is within your Limited-classification scope (personal care, homemaker, companionship — not medication administration or nursing).

Stage 2 — Assessment & the service plan

Do an in-home assessment and build a service plan: the tasks, schedule, and hours, and any safety notes — one of your three required policy sets governs service plans, so follow it.

What to review in the home:

- Mobility and transfer needs; fall hazards; who manages medications; family and other supports; and, for any APD-funded client, the authorized hours your plan must match.

Stage 3 — Agreement & start of care

The client (or representative) signs a service agreement (services, schedule, rate/payer, rights, termination). Match a screened, competent caregiver and start with a supervised introduction.

Before the first shift, confirm:

- The caregiver's background determination and competency evaluation are complete and on file; the client's rights and complaint process were explained; and the service agreement is signed by the client or their representative.

Stage 4 — Delivering the hours

The caregiver delivers care; keep visit notes and protect the client-caregiver match — consistency drives satisfaction and retention.

Stage 5 — Supervision & quality

Supervise per your policy; monitor performance and changing needs; run your quality program; and report any suspected abuse, neglect, or exploitation.

Stage 6 — Changes & reassessment

As needs change, update the service plan and the hours; coordinate with any funder. Keep documentation current.

Stage 7 — Discharge & transition

When care ends, follow your discharge policy, give notice, coordinate with the family and any funder, and document. A good discharge turns families into referral sources.

PART 8

How You Get Paid — Private Pay, LTC Insurance, VA & (limited) Medicaid

In short: An Oregon in-home care agency is mostly a private-pay business, because Medicaid personal care flows to individual homecare workers. Build on private pay, LTC insurance, and the VA.

Your revenue streams, in order of realism for an agency:

Source	What it is	Key fact
Private pay	Client/family pays out of pocket, hourly	Your core market — start on your license; ~\$40/hr in OR (2026)
Long-term-care insurance	The client's LTC policy reimburses in-home care	Bill the family or insurer once benefit triggers are met
Veterans (VA)	VA Homemaker/HHA via the Community Care Network	A real agency channel — contract through TriWest (Region 4)
Medicaid (APD)	Oregon Health Plan in-home services	Mostly flows to individual homecare workers — agencies are the minority path

8.1 Private pay — your core market

The day your license issues, serve private-pay clients — families paying out of pocket, typically billed hourly with a visit minimum. Oregon runs above the national median — around \$40/hour for non-medical in-home care (2026 estimates; verify the current Oregon figure with a CareScout/Genworth cost-of-care report). Set your rate above your fully-loaded caregiver cost.

8.2 Long-term-care insurance

Many long-term-care policies reimburse licensed in-home personal care once the benefit triggers are met (typically needing help with two or more ADLs, or cognitive impairment). Being an Oregon-licensed In-Home Care Agency qualifies you as a covered provider on most policies — you bill the family or the insurer and help with the claim.

8.3 Veterans — a real agency channel

- **VA Homemaker / Home Health Aide & Veteran-Directed Care:** the VA's in-home programs — contract through the VA Community Care Network. Oregon is in Region 4, administered by TriWest (the VA is reorganizing the network — verify the current administrator).
- **Aid & Attendance:** an increased VA pension (verify current amounts on VA.gov) a veteran can apply toward your private-pay care.

8.4 The Medicaid reality

Oregon's Medicaid personal care is dominated by individual homecare workers (paid through the Oregon Home Care Commission) and consumer-directed models like the Independent Choices Program — not agencies. An agency CAN contract with APD to provide in-home services (OAR 411-033) and bill through the state, but most hours flow to homecare workers, and it's a lower-margin channel. Don't build your plan on Medicaid agency revenue; build it on private pay, LTC insurance, and the VA, and treat APD as an optional add-on.

8.5 If you pursue APD Medicaid — the Oregon enrollment path

If you do decide to add the (minority) Medicaid agency channel later, Oregon runs it through Aging & People with Disabilities and its state-built eXPRS system rather than a commercial EVV vendor — so the enrollment path looks different from most states. Walk it in this order, and verify each step with your local APD/AAA office before you rely on it:

1. **Be a licensed In-Home Care Agency first.** Your OHA license (Part 4) is the prerequisite — APD contracts only with licensed agencies, so finish licensure before you start Medicaid enrollment.
2. **Enroll as an Oregon Medicaid provider.** Apply for an Oregon Health Plan provider enrollment and a Medicaid provider number through ODHS/OHA — you'll need your Type 2 NPI (Part 3) to do it.

3. Contract with APD for in-home services (OAR 411-033). Work through your local APD or Area Agency on Aging office to set up the agency in-home services contract and understand the authorization process for member hours.

4. Onboard to eXPRS for authorizations, EVV & billing. Oregon's eXPRS system carries your service authorizations, captures the electronic visit verification the state requires for Medicaid personal care, and is where you submit claims — set up your agency, workers, and clients in it before your first Medicaid visit.

5. Pull the current rate schedule. APD's in-home services rates are set by the state and change periodically (a new schedule typically takes effect each January) — get the live figure from APD or your contract rather than a quoted number.

Why most Oregon agencies wait. *Because eXPRS enrollment, APD contracting, and the state's lower agency rates all take time and pay less than private pay, most new Oregon agencies launch entirely on private pay and LTC insurance and add APD only once they have steady cash flow and a full caregiver roster. There's no penalty for starting private-pay-only.*

PART 9

What It Costs & How Long It Takes

In short: Budget the license, insurance & bonding, caregiver training, payroll float, and (optionally) accreditation. Launch on private pay.

9.1 Startup line items (typical — verify)

Item	What's in it	Ballpark
OHA license (Limited)	Initial In-Home Care Agency license	~\$2,000 (renewal ~\$1,000)
Business setup	SOS LLC \$100 + \$100/yr report, EIN free, NPI free, BIN, legal	~\$200 + legal
Insurance & bond	Liability + abuse + non-owned auto + dishonesty bond + workers' comp	Get quotes — annual
Caregiver training	Orientation + competency + 6 hrs annual per caregiver	Per caregiver
Software	Scheduling / records / invoicing / payroll platform	Monthly SaaS
Payroll float	You pay caregivers before clients/insurers pay you	Working capital
Policies	P&P Manual + Forms Packet	One-time
Accreditation (optional)	ACHC / CHAP / Joint Commission	~\$7,000–\$14,500 / 3-yr cycle

The cash-flow trap to plan for. You pay caregivers before LTC insurers and the VA pay you. Private-pay clients (billed in advance or on short terms) smooth this out — line up a business line of credit before you scale.

9.2 Timeline

Weeks	Entity & policies	Form the LLC, write your three policy sets, line up your administrator
Weeks	Insurance & background	Bind coverage & bond; start background checks
Submit	OHA application	Application + fee (Limited); prepare for the survey
Survey	Pre-licensure survey	Pass the on-site survey; license issues
Right away	Start private pay	Serve private-pay & LTC-insurance clients
Then	Add VA	Contract with TriWest (CCN); optionally add APD

9.3 The money math — a simple break-even

Home-care economics are simple once you see them laid out. On each private-pay hour, you bill the client a rate; out of that you pay the caregiver's wage plus payroll taxes, workers' comp, and your overhead. The gap is your gross margin — often roughly 30–40% of the bill rate for a well-run private-pay agency. A worked example, using round numbers you'll replace with your own:

- **Bill rate to the client:** say \$40/hour (set yours from your local market and what referral sources see).
- **Fully-loaded caregiver cost:** the wage (say \$15) plus about 15–20% for payroll taxes, workers' comp, and paid time — roughly \$18/hour all-in.
- **Gross margin:** about \$12/hour, before overhead — from which you cover insurance, software, office, marketing, and your own pay.
- **What this means for you:** you need enough billed hours each week to cover your fixed overhead before you earn a profit — which is why census (total authorized client hours) and caregiver utilization are the two numbers to watch, and why a few reliable, long-hours clients are worth more than many tiny visits.

9.4 The hidden costs new owners miss

The line items above are the visible costs. The ones that surprise underprepared owners are the invisible ones — plan for these before you open:

- **Payroll float:** you pay caregivers every week or two, but LTC insurers and the VA can pay you 30–60 days after you bill. Keep several weeks of payroll in reserve, or lean on private-pay clients (who pay up front or on short terms) to smooth cash flow.
- **Unbillable overhead:** your own time, a scheduler/coordinator, office and phone, background checks, and caregiver supervision — none of it is billable, all of it is real.
- **Turnover churn:** every caregiver who quits costs you re-hiring, re-screening, and re-training. Budget for it, and spend on retention instead — it's cheaper than replacement.
- **Pre-licensure delay:** Oregon's survey and high application volume mean months with costs but no revenue. Line up private-pay referrals so you can start the day your license issues.

PART 10

Getting Your First Clients

In short: *Because the business is private pay, clients come through referral relationships — discharge planners, home-health/hospice partners, the ADRC/AAAs, care managers, and senior communities. Build these before you open.*

10.1 The referral sources

- **Hospital discharge planners & case managers:** the top source for “needs help at home now” — make same-day starts easy.
- **Home-health & hospice partners:** they refer the non-skilled personal-care hours their benefit doesn't cover; build reciprocal partnerships.
- **The ADRC of Oregon & Area Agencies on Aging:** the statewide Aging & Disability Resource Connection (1-855-673-2372) and the regional/Type B AAAs (often Councils of Governments) field families needing in-home help — get listed and known.
- **Geriatric care managers & senior communities:** Aging Life Care professionals and assisted-/independent-living communities and CCRCs refer supplemental one-on-one care.
- **Oregon 2-1-1 (211info):** list your services (211info.org) — a free statewide referral channel.
- **Elder-law attorneys & fiduciaries:** they advise families arranging and funding care.

10.2 What to ask each referral source

“How do you decide which home-care agency to refer a family to?”

“What do you need from me to be on your referral or preferred-provider list — license, insurance, bonding, Medicaid enrollment, availability?”

“Who exactly is the discharge planner, social worker, or case manager I should build a relationship with?”

“How fast do you need us to be able to start care after a referral — same day, 24 hours, 48 hours?”

“What makes an agency easy to work with, and what gets one dropped from your list?”

Speed and reliability win home-care referrals more than anything else: be the agency that answers the phone on the first call, can start care within 24–48 hours, communicates back to the referrer, and never no-shows a shift. One reliable start earns you the next ten referrals.

10.3 Your first-90-days referral plan

Referrals don't happen by accident — they come from a handful of relationships you build on purpose. Here's a simple plan for your first 90 days:

1. **Make a target list.** Write down the 10–15 hospital discharge planners, skilled home-health and hospice liaisons, senior-community directors, and case managers in your service area — these are the people who send families to agencies.
2. **Show up in person with a one-pager.** A clean one-page sheet: who you are, what you do, your service area, how fast you can start, your insurance/credentials, and your phone number. Leave it; then follow up a week later.
3. **Be absurdly responsive.** Answer every call, say yes to same-day starts whenever you can, and report back to the referrer after the first visit — that single habit earns the next referral.
4. **Ask for feedback and the next referral.** After a good start, ask the referrer what would make you easier to work with, and whether they have another family who needs help. Then do it again next week.

PART 11

Who to Call & Exactly What to Ask — Every Verify, in One Place

In short: Your master contact + verification hub. Everywhere the guide says “verify,” the who and the what-to-ask are collected here.

Licensing, the administrator & staff

Who	Contact	What to verify / ask
OHA — HCRQI (In-Home Care)	971-673-0540 · mailbox.inhomecare@odhaoha.oregon.gov	The application & classification (Limited); the current fee; the pre-licensure survey
ODHS/OHA Background Check Unit	oregon.gov/odhs/background-checks	The background-check/fitness determination
Caregiver training resources	your own program / vendors	OAR 333-536-0070 orientation, competency, 6-hr annual
Legal counsel	an Oregon home-care attorney	The classification choice; the referral/matchmaker licensure trap

Business, insurance & billing IDs

Who	Contact	What to verify / ask
Oregon Secretary of State	sos.oregon.gov/business	Forming your LLC (\$100); the \$100 annual report
Oregon Dept. of Revenue	oregon.gov/dor	Your Oregon BIN (payroll-tax registration)
IRS	irs.gov (EIN Assistant)	Get your EIN (free)
NPPEs / CMS	npes.cms.hhs.gov	Your Type 2 NPI (for VA/APD billing)
Insurance agent (home-care program)	trustedchoice.com; carriers in Part 3.3	Liability + abuse + non-owned auto + dishonesty bond + workers' comp

Funding & referrals

Who	Contact	What to verify / ask
VA Community Care / TriWest (Region 4)	vacn.triwest.com	The VA Homemaker/HHA program; joining the CCN; the current administrator
ODHS/APD (Medicaid, if you pursue it)	oregon.gov/odhs	Agency in-home services under OAR 411-033; eXPRS; the Jan 2026 rate schedule
ADRC of Oregon	1-855-673-2372 · adrcforegon.org	Getting listed; local referral partners
Long-term-care insurers	the client's policy / carrier	Benefit triggers; whether a licensed agency is required; how to bill

Referral & senior-services partners

Who	Contact	Role
Area Agencies on Aging / Type B AAAs	via the ADRC	Local information & referral for older adults
Oregon 2-1-1 (211info)	dial 211 · 211info.org	Free statewide referral line — list your services
Accreditor (optional) — ACHC/CHAP/TJC	achc.org · chapinc.org · jointcommission.org	Whether/when to accredit; VA/payer requirements
Hospital / SNF discharge planners	your local hospitals & SNFs	The highest-volume private-pay referral source

Common questions, answered

Do I have to take Medicaid?

No. Many successful agencies run entirely on private pay, long-term-care insurance, and veterans' benefits — it's often the smarter way to start, because you earn immediately without the enrollment, contracting, and EVV that Medicaid requires. You can add Medicaid later once you have cash flow and a team.

Can I run this from home at first?

Often yes for a small non-medical agency — the care happens in the client's home, so you may not need a storefront to start. Confirm your local zoning and your license/registration rules (some require a business address), and know you'll want real office space as you grow.

Should my caregivers be employees or independent contractors?

Almost always W-2 employees. Classifying caregivers as 1099 contractors to save on taxes is one of the most common — and most expensive — mistakes in home care; it invites wage-and-hour and tax liability, and most payers and insurers require employees. Treat them as employees and carry workers' comp.

How much cash do I really need to start?

Enough to cover several weeks — ideally a couple of months — of caregiver payroll before your first payer reimbursements arrive, plus your license/insurance/software setup. Under-capitalization is the number-one reason new agencies fail; line up a business line of credit before you scale.

How long until it's profitable?

Most agencies take several months to a year to build a steady client base and referral flow. Starting on private pay shortens the runway because you're paid quickly; Medicaid volume comes later and pays slower. Plan your finances for a ramp, not an overnight full census.

Do I need experience in health care?

It helps, but plenty of successful owners come from business, sales, or family-caregiving backgrounds and hire the clinical expertise they need (for example, a nurse where the state requires one). What matters more is being organized, responsive, good with people, and disciplined about compliance and cash.

What actually makes one agency win over another?

Reliability. Families and referral sources choose — and stay with — the agency that answers the phone, starts care fast, sends the same dependable caregiver, communicates, and never leaves a shift unstaffed. Get that right and referrals compound; get it wrong and no amount of marketing saves you.

Do I need a nurse on staff in Oregon?

Not for a non-medical agency. Oregon's In-Home Care Agency license has four classifications, and the Limited (and Basic) tiers — personal care, homemaker, and companionship — require no RN. A nurse is only required for the Comprehensive/nursing classification. You can start Limited and move up a tier later if you ever add clinical services.

Why is Oregon mostly a private-pay market?

Because Oregon funnels most Medicaid personal care to individual homecare workers paid through the Oregon Home Care Commission, not to agencies. An agency can contract with APD (Part 8.5), but it's the minority, lower-margin path — so a well-run Oregon agency builds on private pay, long-term-care insurance, and the VA first.

You can do this

Opening an Oregon in-home care agency is very doable — one license, no nurse required for the Limited tier, and a strong private-pay market. Go in clear-eyed that it's a private-pay business (Medicaid flows to homecare workers, not agencies): form the business, get the Limited license, recruit and train great caregivers, launch on private pay and LTC insurance, and add the VA. Verify each fee and contact as you go, and lean on the people in Part 11.

Reminder. Prepared for PolicyReady.org and grounded in ORS 443.305–443.350, OAR chapter 333 Division 536, and current Oregon information as of 2026. This is general information, not legal, tax, or billing advice. Requirements, fees, rates, and contacts change — verify each with OHA and qualified counsel before you rely on it. Confirm the current license fee and classification requirements before you rely on them.